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RURAL HEALTH TRANSFORMATION

State of Idaho Rural Health Transformation Program

Idaho Department of Health and Welfare

SOLICITATION FOR SUBAWARD APPLICATIONS

Chronic Disease Prevention and Cancer Screenings

Federal Agency	Centers for Medicare & Medicaid Services
Federal Award No.	RHTCMS332084
ALN No.	93.798
Solicitation No.	RHTPDIS1
Posting Date	08/11/2026
Application Deadline	09/08/2026
Estimated Award Range	\$1,000,000– \$2,000,000 per award.
Anticipated Project Period	09/30/2026 – 09/30/2027
Point of Contact	IdahoRHTP@dhw.idaho.gov

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Section 1: Program Description & Purpose

1.1 Background

The Department will serve as the lead agency for administering the Rural Health Transformation Program (RHT) Grant funding award for the state of Idaho (ID). As such, the Department is committed to removing barriers to treatment, improving health outcomes, and promoting patient-centered care. The RHTP website address is: <https://healthandwelfare.idaho.gov/providers/rural-health-transformation-program-grant/about-rural-health-transformation-program-grant>

As the grant recipient, the Department manages the entire grant award and is responsible for distributing funding to other ID state agencies, contractors, and Subgrantees in alignment with the projects awarded by Centers for Medicare & Medicaid Services (CMS). This distribution will be carried out in accordance with both state and federal regulation, specifically procurement rules, to facilitate the execution of RHTP work. The state's RHTP is a critical mechanism to advance a sustainable, data-driven model of care that enhances access, quality, and outcomes.

Idaho's Rural Health Transformation Plan (RHTP) (Attachment A, ID RHTP Project Narrative) is a multi-year strategic plan including five (5) major initiatives designed to strengthen rural health systems, increase access to care, enhance quality outcomes, expand workforce capacity, and promote financial sustainability. These initiatives address CMS' strategic goals for this grant to transform healthcare for residents in the state's rural areas.

The five initiatives are:

Initiative 1. Improving rural access to care through technology

- Facility technology assessments and shared infrastructure
- Telehealth expansion
- Digital health
- Cybersecurity modernization and artificial intelligence (AI) tools
- Emergency communication systems implementation and enrollment
- Health management and data analytics tools
- EHR software and upgrades

Initiative 2. Ensuring accessible, quality care through innovative models

- Diagnostic and care access innovations
- Health extenders
- Emergency medical service (EMS) expansion and workforce stabilization
- Understand opportunities to leverage health extenders and optimize service delivery

Initiative 3. Sustaining rural workforce with training, recruitment, and retention.

- Ladder payments based on priority positions and rural presence
- Healthcare career exploration and advancement programs
- Work-based learning

- Graduate medical education (GME) programs

Initiative 4. Implementing population specific, evidence-based projects to Make America Healthy Again

- Chronic disease prevention and treatment
- Behavioral health prevention and treatment
- Maternal and child health efforts

Initiative 5. Investing in rural health infrastructure and partnerships

- Healthcare facility renovations
- Pharmacy solutions
- Clinical equipment purchases
- Vehicles for patient transport and rural mobile health units
- Healthcare facility federal, state, and local safety code compliance
- Tribal rural health transformation support

The state's RHTP leverages partnerships with state and local agencies, tribal partners, healthcare providers, and community organizations to ensure locally responsive solutions.

This plan will work to modernize electronic and logistical infrastructure, strengthen community-to-clinician connections, and further integrate healthcare and community-based services across settings and types of care.

1.2 Project Coordination

To ensure alignment across the Idaho Rural Health Transformation initiatives and to prevent duplication or overlap, the selected applicant must, as applicable to the scope of work, coordinate with Department staff, other Rural Health Transformation-funded entities, and key stakeholders in conducting assessment, data collection, evaluation, reporting, and development of deliverables.

1.3 Program Goals & Objectives

This solicitation specifically addresses RHTP Initiative 4: **Implementing population specific, evidence-based projects to Make America Healthy Again**. The goal of this funding is to reduce incidence and improve the management of chronic diseases through proven prevention, screening, and education programs tailored specifically for rural populations. Activities funded through this solicitation are encouraged to recognize and engage family caregivers when appropriate to support improved health outcomes.

The Idaho Department of Health and Welfare (Department) seeks to award up to two (2) subgrantees to establish, administer, and oversee statewide chronic disease prevention and management initiatives and cancer screening initiatives. The successful applicant(s) may serve as pass-through organizations to local partners for implementation.

Required Activities:

- Implement evidence-based chronic disease prevention and management programs in rural Idaho.
- Expand access to chronic disease and cancer screening, education, and self-management supports.
- Build sustainable community partnerships.
- Integrate technology to overcome rural access barriers.
- Improve measurable health outcomes among rural Idahoans.

Program Deliverables: Subgrantees must submit the following deliverables (at minimum):

- Work plan with milestones and timelines (updated quarterly).
- Evidenced-based curriculum delivery logs, including fidelity documentation.
- Monthly attendance and enrollment logs for all programs implemented.
- Technology utilization dashboards: logins, telehealth visits, device activation.
- Devise distribution and inventory reports.
- Documentation of community education events (location, reach, demographics).
- Quarterly report summarizing screening activities and referrals completed.

1.4 Scope of Work

Successful applicants must work with organizations who serve rural populations to implement targeted interventions that address **cancer, heart disease, and other chronic conditions**. Applicants must fulfill the following approved activities:

- A. Partnerships and Capacity Building:** Coordinate and partner with the Department, hospitals, Federally Qualified Health Centers (FQHCs), Emergency Medical Service (EMS) providers, tribal entities, local public health districts and community organizations to avoid duplication, build upon existing statewide initiatives, lessons learned, and available data, and ensure rural alignment.
 - a. Participate in RHTP meetings and statewide evaluation activities
 - b. Share data with the Department for reporting, learning collaboratives and quality improvement
 - c. Align work with existing Regional Health Needs Assessments and Community Health Improvement Plans, Department priorities, and other relevant statewide planning efforts, as appropriate
 - d. Incorporate family caregivers and care partners into planning and referral processes and facilitate connections to family caregiver support resources when appropriate
 - e. Collaborate with the Department to incorporate existing statewide initiatives, implementation experience, available data, and lessons learned into project planning, implementation, and continuous quality improvement activities.
- B. Evidence-Based Programming:** Implement one or more nationally recognized evidence-based intervention, such as:
 - a. Dietitian-led nutrition program.
 - b. Structured physical activity classes.
 - c. Lifestyle behavior-change series.
 - d. Community-based chronic disease self-management.
 - e. Community-based caregiver support programs.
 - f. Establish formal referral pathways to support coordinated transitions between screening, diagnosis, evidence-based interventions, and caregiver supports.

- C. Screening, Early Detection and Referral Expansion:** Increase access to chronic disease and cancer screenings by:
- Offering mobile or community-based screening events in rural settings.
 - Partner with rural healthcare clinics, FQHCs, and/or hospitals to expand screening capacity.
 - Use evidence-based risk assessments and referral pathways that align with existing statewide resources and Department-supported initiatives, when applicable.
 - Integrate electronic referral or follow-up systems into business procedures, electronic health record, or other patient care management processes.
- D. Technology Integration and Digital Health Tools:** Deploy scalable, technology-based programs and tools to promote sustainable health habits and overcome rural healthcare access barriers. Programs and tools include, but are not limited to:
- Telehealth platforms for coaching, education, and follow-up.
 - Remote patient monitoring tools (blood pressure cuffs, glucometers, scales, etc.).
 - Mobile health apps for self-management tracking.
 - Digital screening and referral tools.
- E. Community Education, Outreach and Health Promotion:** Increase rural awareness and education for rural Idahoans and their caregivers through:
- Regular community workshops focused on chronic disease risk, nutrition, physical activity, cancer prevention and heart health.
 - Small group coaching sessions.
 - Community-wide campaigns delivered through rural radio, print, TV, social media, and physical locations such as schools, senior centers, libraries, businesses, etc.
- F. Evaluation:** The Subgrantee must meet at least quarterly with the Department's Rural Health Transformation Program Data Analytics and Evaluation Contractor to complete the following tasks:
- Develop theory of change diagram(s) (e.g., logic model, line-of-sight diagram, driver diagram, or similar visualization of assumptions connecting proposed work with intended outcomes and performance indicators)
 - Use data sharing mechanisms (to be developed by the Data Analytics and Evaluation contractor) to report on required key performance objectives (KPO) within timelines established by the Department which may include, but are not limited to:
 - Number of sites implementing new or enhanced evidence-based prevention or treatment programs
 - Number of individuals enrolling in a new or enhanced evidence-based prevention or treatment program
 - Number of individuals reaching program specific retention or completion milestone
 - Number of individuals who reach program specific clinical outcome (e.g., A1c, weight reduction, hypertension control)
 - Other measures to be defined in consultation with the Data Analytics and Evaluation contractor after award
 - Develop and implement a process evaluation and quality improvement plan that incorporates project findings, Department feedback, statewide lessons learned, and useful evaluation or process improvement measures in addition to required KPOs.
 - Review and provide input on draft evaluation findings, reports, and analyses as requested.
- G. Non-Duplication Assessment:** The subgrantee must complete the Non-Duplication Assessment (Appendix H) and submit with application to ensure funds do not support an existing project or plan.

1.5 Target Population & Service Area

The ultimate beneficiaries of this solicitation include Idahoans living in rural and frontier communities who are at risk for or living with chronic health conditions, including cardiovascular disease, stroke, cancer, and obesity.

Family caregivers and care partners may also benefit from activities supported through this solicitation. Family caregivers often play an important role in supporting prevention activities, treatment adherence, care coordination, self-management, and long-term disease management. Workforce education activities should promote awareness of caregiver engagement practices and available caregiver resources when appropriate.

Geographic Focus and Service Area:

Eligible service areas are rural and frontier counties within the state of Idaho, as defined by state rural health criteria and federal rural-urban classification systems, including non-metropolitan counties and rural census tracts designated under USDA Rural-Urban Continuum Codes (RUCC) or the Federal Office of Rural Health Policy (FORHP) eligibility criteria.

See Appendices A and B for the RHTP eligibility map and a list of eligible and ineligible cities by county.

Target Audience: Individuals residing in rural areas who are at risk for, or are currently managing, chronic conditions such as cancer, heart disease, stroke, and obesity.

Section 2: Eligibility Requirements

2.1 Eligible Applicant Types

Eligible entities include:

- State/local government agencies
- Tribal organizations
- Nonprofit organizations with 501(c)(3) status
- Institutions of higher education located in Idaho
- Healthcare associations and professional societies
- Hospitals and health systems in Idaho
- Federally qualified health centers (FQHCs).
- Organizations with demonstrated experience administering statewide clinical education, workforce development, continuing education, learning collaboratives, or healthcare provider training initiatives.

2.2 Minimum Qualifications

- Applicants must demonstrate experience administering statewide or multi-regional healthcare workforce development, continuing education, learning collaborative, tele-mentoring, clinical education, practice improvement, or pass-through funding programs involving multiple participating organizations.
- Active registration in SAM.gov with a current, valid Unique Entity Identifier (UEI)
- No active exclusions in SAM.gov or on the HHS Office of Inspector General Exclusion List

- Demonstrated experience (minimum 2 years) managing federal or state-funded health programs
- Financial management systems capable of separately tracking and reporting federal expenditures per §200.302
- If the applicant expended \$1,000,000 or more in federal awards in their most recent fiscal year, a completed Single Audit per §200.501 with no unresolved material findings
- Current organizational insurance coverage including general liability and professional liability

Because Lead Entities will administer subgrants and serve as pass-through entities, applicants must also demonstrate:

- Written policies and procedures governing subaward administration
- Internal control systems that comply with 2 CFR 200.303
- Experience conducting pre-award risk assessments
- Experience monitoring subrecipients through desk reviews, site visits, report reviews, and corrective action processes
- Capacity to review and approve reimbursement requests and support documentation
- Experience issuing management decisions and corrective action plans when compliance concerns are identified
- Systems for collecting, validating, and aggregating performance data from multiple implementation partners
- Experience managing statewide or multi-site initiatives involving multiple stakeholders.

2.3 Exclusions

The following entities are not eligible to apply for funding under this subgrant solicitation:

- Organizations that are debarred, suspended, or otherwise excluded from participation in federal programs, as verified through the System for Award Management (SAM.gov)
- Applicants that do not maintain active registration in the System for Award Management (SAM.gov)
- Applicants with unresolved audit findings, including outstanding single audit findings or material weaknesses that have not been adequately addressed to the satisfaction of the awarding agency
- Applicants that do not have the financial management systems and internal controls necessary to manage federal funds in accordance with applicable requirements
- Individuals applying in a personal capacity

The awarding agency reserves the right to verify eligibility and disqualify applicants that do not meet federal and state compliance requirements.

2.4 Cost Sharing / Match Requirements

No cost share or match is required for this subaward. However, applicants who can demonstrate leveraged resources (e.g., in-kind staff time, donated facilities, complementary funding streams) are encouraged to describe these in the Sustainability section of their Project Narrative, as they strengthen the overall application.

Section 3: Award Information

3.1 Funding Details

Total Available Funding	\$2,000,000
Anticipated No. of Awards	1-2
Award Range (per subaward)	\$1,000,000-\$2,000,000
Subaward Type	Reimbursement
Period of Performance	9/30/2026-9/30/2027
Continuation Funding	No

3.2 Indirect Cost Rate

As required by the Centers for Medicare & Medicaid Services under Rural Health Transformation, administrative costs, including indirect costs, are limited to 10 percent of the total approved subaward budget. Indirect costs must be calculated on the Modified Total Direct Cost (MTDC) base as defined in 2 CFR 200.1.

Together, indirect costs and direct administrative costs may not exceed 10 percent of the total approved budget. This federally required limitation applies to all subawards. Costs exceeding this threshold are not allowable and will not be reimbursed.

Section 4: Application Instructions

4.1 Required Application Components

Applicants must submit all of the following components as a single, consolidated application package:

#	Component	Format	Page Limit	Required?
1	Cover Page / Application Face Sheet	Template provided (Appendix C)	1	Yes
2	Organizational Capacity Statement	PDF	1	Yes
3	Project Narrative	Template provided (Appendix D)	15 pages	Yes
4	Work Plan with Milestones & Timeline	Template provided (Appendix E)	No limit	Yes
5	Budget & Budget Justification	Template provided (Excel File)	No limit	Yes

6	Letters of Support / MOUs	PDF	No limit	If applicable
7	Proof of SAM.gov Registration / UEI	Screenshot or PDF	N/A	Yes
8	Current Indirect Cost Rate Agreement	PDF	N/A	If applicable
9	Most Recent Single Audit (if >\$1 million)	PDF or link	N/A	If applicable
10	Pre-award Financial Questionnaire	Template provided (Appendix F)	N/A	Yes
11	Signed Certifications & Assurances	Template provided (Appendix G)	N/A	Yes
12	Assurance of Non-Duplication	Template provided (Appendix H)	N/A	Yes

4.2 Project Narrative Guidance

The Project Narrative is the primary basis for evaluating the applicant's approach and shall not exceed fifteen (15) pages. Address each area below in the order listed. Reviewers will evaluate narrative quality, specificity, and alignment with program goals. Vague or generic responses will score poorly.

- Statement of Need:** What specific chronic disease does the proposed project address through prevention and appropriate disease management? Describe the specific chronic disease burden in your service area using local, regional, or state data. Identify the populations most affected and the root causes driving disparities. Explain why current resources and programs are insufficient and how this subaward will address unmet needs. Support with data.
- Goals & Objectives:** What measures will be tracked to demonstrate improved health outcomes; reduced disease burden; and reduced cost of care (for individuals, providers, community, state, etc.)? Use SMART (Specific, Measurable, Achievable, Relevant, Time-bound) criteria. For each objective, identify a baseline value, a target value, and the data source you will use to measure progress.
- Approach & Methods:** How will you accomplish the objectives? Describe your implementation plan in detail. Describe interventions, evidence base, and innovation. For each proposed activity, explain: the evidence data supporting the intervention, how you will adapt it to your local context, the implementation timeline, who will deliver the intervention, and how you will ensure fidelity to the model.
- Target Population & Engagement:** Who will be served and how will you ensure access in a rural community? Describe the population(s) you will serve, including demographics, geographic distribution, and how you determined community priorities. Describe your approach to community engagement, including how community members will be involved in program design, implementation, and evaluation. If serving tribal communities, describe your tribal consultation process and any existing agreements.

- **Partnerships & Collaboration:** What organizations will you work with and what roles will they play? Identify key partner organizations and describe each partner's role, contribution, and the formal or informal agreement governing the partnership. Explain how partnerships will enhance program reach, reduce duplication, and support sustainability. Include letters of support or MOUs from each named partner as an attachment.
- **Evaluation Plan:** How will you measure progress and outcomes? What data will be collected? Describe your plan to evaluate process outcomes (reach, dose, fidelity) and health outcomes (clinical measures, behavior change). Identify data collection methods, tools, frequency, and responsible staff. Explain how you will use evaluation data for continuous quality improvement during the funding period.
- **Sustainability:** How will activities or outcomes continue beyond the funding period? Describe how key program activities, partnerships, or outcomes will be sustained after the subaward ends. Identify potential future funding sources, organizational commitments, and policy or systems changes that will outlast the grant.

4.3 Budget & Budget Justification Guidance

Applicants must submit a detailed, line-item budget for the full period of performance and a narrative justification for each cost category. The budget must demonstrate that all proposed costs are allowable, allocable, reasonable, and necessary per 2 CFR 200 Subpart E (§§200.400–200.476).

- **Personnel:** List each position by title, annual salary, % FTE charged, and fringe benefit rate. Justify each position's necessity to the project.
- **Fringe Benefits:** Provide the fringe rate and its components (FICA, retirement, health insurance, etc.). If rates differ by employee class, break them out separately and display in a table.
- **Travel:** Itemize trips by purpose, destination, number of travelers, and estimated costs (airfare, mileage at the federal rate, lodging, per diem).
- **Equipment:** Items with a per-unit cost of \$2,000 or more and a useful life exceeding one year. Prior approval from the Department is required for all equipment purchases.
- **Supplies:** Items under \$2,000 per unit. Group by category (office supplies, program materials, technology) and justify quantities.
- **Contractual:** Describe each proposed contract or subaward, including the scope, estimated cost, and procurement method. Note: subrecipients may not subaward to other entities without prior written approval from the Department.
- **Other Direct Costs:** Includes participant incentives (limited to \$25 per participant per encounter, \$150 max per participant per year), printing, communications, training fees, software licenses, etc.
- **Administrative Costs:** No more than 10% administrative costs.

Payment and Reimbursement

Subawards will be issued on a reimbursement basis. Lead Entities will be responsible for:

- Reviewing and approving reimbursement requests submitted by local implementation partners

- Maintaining all supporting documentation
- Ensuring all expenditures comply with applicable federal and state requirements
- Submitting reimbursement requests and required supporting documentation to the Department

Payments are contingent upon satisfactory performance, timely reporting, and compliance with all award requirements.

4.4 Submission Instructions

Submission Method	Email
Submit To	IdahoRHTP@dhw.idaho.gov
Deadline	09/08/2026
Late Policy	Late applications will not be reviewed.
Questions Deadline	08/19/2026 – all questions must be submitted by this date
Q&A Publication	Responses posted to URL by 08/31/2026

Section 5: Application Review & Evaluation Criteria

5.1 Evaluation Criteria & Scoring

Criterion	Max Points	Key Considerations
Statement of Need & Alignment with Program Goals	10	Defined community need indicators, aligned to NOFO
Population Outcomes, Goals, Objectives & Work Plan	15	SMART, feasible, measurable
Statewide Network Administration and Subgrant Management Capacity	20	Prior subgrant administration, monitoring procedures, fiscal oversight system
Technical Assistance and Capacity-Building Approach	10	Expertise, time commitment
Fiscal and Programmatic Monitoring Capacity	10	Clearly defined monitoring procedures, fiscal oversight system
Organizational Capacity and Relevant Experience	10	Track record, infrastructure, staffing
Evaluation, Data Collection, and Reporting Systems	10	Data-driven methods, tools, reporting capability, data management plan
Budget and Cost Effectiveness	10	Allowable, allocable, reasonable
Sustainability & Partnerships	5	Continuation plan, MOUs
TOTAL	[100]	

5.2 Review Process

The review process consists of three stages:

- **Administrative Screening:** Department staff will screen applications for completeness and eligibility. Incomplete applications or those from ineligible organizations will not advance to technical review. Applicants will be notified of disqualification within 5 business days.
- **Technical Review:** A review panel of at least three individuals with expertise in chronic disease prevention, public health program management, and federal grants compliance will independently score each application. Reviewers will participate in a consensus scoring session to discuss and finalize scores. Panel members will sign conflict of interest disclosures.
- **Final Selection:** The RHTP Director or DHW Deputy Director will make final funding decisions based on panel scores, geographic coverage across the state, population reach, and programmatic priorities. The Department reserves the right to fund applications out of rank order to ensure equitable geographic distribution.

The Department may request clarification, additional information, or interviews from applicants as part of the review process. Funding recommendations will be developed based on final evaluation scores, risk assessment results, available funding, and the Department's goal of increasing chronic disease prevention and management initiatives, including cancer screenings, in Idaho. The Department anticipates funding one (1) to two (2) Lead Entities capable of administering and overseeing a statewide network of local implementation partners.

- Final funding decisions will consider:
- Application scores
- Funding availability
- Geographic coverage
- Organizational capacity
- Program priorities
- Ability to achieve statewide population-level outcomes

5.3 Pre-Award Risk Assessment

All recommended applicants will undergo a pre-award risk assessment prior to subaward execution. This assessment will evaluate:

- Prior experience managing federal awards of similar size and scope
- Results of previous audits, including Single Audit findings (if applicable)
- Ability to adhere to federal statutes, regulations, and subaward terms
- Financial stability and adequacy of financial management systems
- Adequacy of internal controls and policies (procurement, property, travel, etc.)
- Current or pending corrective action plans

Applicants who do not pass the pre-award risk assessment may have their award rescinded or may receive the award with special conditions (e.g., more frequent reporting, limited advance payments, required technical assistance).

Section 6: Subaward Terms & Conditions

6.1 Federal Flow-Down Requirements

Subgrantees will be subject to all applicable federal requirements that flow down from the prime award, including but not limited to:

- 2 CFR Part 200 – Uniform Administrative Requirements, Cost Principles, and Audit Requirements
- 45 CFR Part 75 (HHS-specific implementation, if applicable)
- Federal award terms and conditions as specified in the Notice of Award
- CMS/CDC/HRSA program-specific requirements from the Notice of Funding Opportunity (NOFO)
- Single Audit requirements per §200.501 (for entities expending \$1,000,000+ in federal awards)

6.2 Reporting Requirements

Subgrantees will be required to submit the following reports:

Report	Frequency	Due Date
Operating and Personnel Detail Reports Submitted with Invoice	Monthly	15 of following month
Reimbursement Requests and Supporting Documentation	Monthly	15 of following month
Financial Status Report (FSR)	Quarterly	30 after quarter end
Program Progress Report	Quarterly	30 after quarter end
Performance Measures / Data	Quarterly	30 after quarter end
Subgrant Monitoring Report	Quarterly	30 after quarter end
Sustainability Progress Update	Annually	Department designated date
Annual Inventory Report (if applicable) (§200.312)	Annually	[Date]
Final Report (Financial + Programmatic)	End of Period of Performance (POP)	90 days after POP

6.3 Monitoring & Oversight

The pass-through entity will monitor Subgrantee activities to ensure compliance with the subaward terms and applicable federal requirements. Monitoring activities may include:

- Desk reviews of financial and programmatic reports
- On-site or virtual monitoring visits (at least annually for high-risk Subgrantees)
- Review of single audit reports per §200.332(d)
- Technical assistance and capacity-building support
- Review of Subgrantee procurement documentation for compliance with §§200.318–200.327

6.4 Remedies for Noncompliance

If a Subgrantee fails to comply with the terms of the subaward or applicable federal requirements, the pass-through entity may take one or more of the following actions per §200.339:

- Temporarily withhold payments pending corrective action
- Disallow all or part of the cost of the noncompliant activity
- Wholly or partly suspend or terminate the subaward
- Initiate suspension or debarment proceedings per 2 CFR Part 180
- Withhold further awards or impose additional conditions

6.5 Record Retention

Subgrantees must retain all financial records, supporting documents, statistical records, and all other records pertinent to the subaward for a period of six (6) from the date of submission of the final expenditure report, or as otherwise required by §200.334. If any litigation, claim, or audit is started before the expiration of the retention period, records must be retained until all issues are resolved.

6.6 Procurement Standards

Subgrantees must follow the procurement standards in §§200.318–200.327, including conducting all procurement transactions in a manner providing full and open competition. Subgrantees must maintain written conflict of interest policies and document the procurement process including cost/price analysis.

Section 7: Key Dates & Timeline

Milestone	Date
Solicitation Released	08/11/2026
Pre-Application Webinar / Informational Session	08/26/2026; Details will be posted on Funding Opportunities website
Deadline for Questions	08/19/2026
Q&A Responses Published	08/31/2026
Application Deadline	09/08/2026
Review Period	09/10/2026-09/17/2026
Award Notifications	09/28/2026
Anticipated Subaward Start Date	10/19/2026

Section 8: Post Award Anticipated Timeline

Following award execution, Lead Entities will be expected to complete the activities and deliverables in phases.

Phase 1: Startup and Network Development Months 1–3

Phase 2: Subgrant Solicitation and Awards Months 3–6

Phase 3: Program Implementation Months 6–24

Phase 4: Sustainability and Project Closeout Final 6 Months of Performance Period

Ongoing Requirements Throughout the Project Period

Lead Entities shall maintain ongoing compliance with all reporting, monitoring, and performance requirements throughout the project period, including:

- Monthly reimbursement requests
- Quarterly financial reports
- Quarterly programmatic reports
- Quarterly performance measure reporting
- Subgrant monitoring activities
- Participation in Department-led meetings and evaluation activities
- Continuous quality improvement activities

The Department reserves the right to modify timelines, reporting schedules, or milestone expectations as necessary to ensure successful implementation of the statewide chronic disease prevention and management initiatives, including cancer screening.